# 8.12 Syndrome of Inappropriate ADH (SIADH)

| Assessment | Nursing Diagnosis | Plan / Expected Outcome | Nursing Interventions | Evaluation |
|---|---|---|---|---|
| **Subjective**<br>• Family say he is confused<br>**Objective**<br>• $Na^+$ 122 mmol/L<br>• Urine concentrated<br>• Gaining weight with no edema | **ACTUAL**<br>**Excess fluid volume** related to water retention as evidenced by hyponatremia and weight gain. | Sodium comes back up to normal slowly. | • Restrict fluids as prescribed.<br>• Daily weight and a strict I&O chart.<br>• Correct the sodium slowly, as ordered. | Sodium rising: **goal met**. |
| **Subjective**<br>• Family say<br>• "He is confused."<br>**Objective**<br>• Disoriented | **ACTUAL**<br>**Acute confusion** related to hyponatremia as evidenced by disorientation. | Oriented as his sodium corrects. | • Neuro checks every 4 hours.<br>• Reorient him often.<br>• Report any seizure. | Oriented: **goal met**. |
| **Subjective**<br>• *No specific complaint*<br>**Objective**<br>• Sodium very low<br>• Confused | **RISK**<br>**Risk for injury** as evidenced by risk factors: seizures and falls from low sodium. | No injury. | • Seizure precautions in place.<br>• Fall precautions in place.<br>• Bed kept low. | No injury: **goal met**. |